PREOPERATIVE DIAGNOSIS: Chronic suppurative otitis media with large central tympanic membrane perforation, left ear, with conductive hearing loss.

POSTOPERATIVE DIAGNOSIS: Central tympanic membrane perforation, left ear.

PROCEDURE: Left lateral-graft tympanoplasty with temporalis fascia autograft.

OPERATIVE TECHNIQUE: Under general anesthesia with endotracheal intubation, the patient was positioned with the head turned to the right. The left auricle and postauricular area were prepped and draped in sterile fashion. A postauricular incision was made 5 mm posterior to the postauricular crease. Dissection was carried through the subcutaneous tissues to expose the temporalis muscle fascia. A 2.5 x 2.5 cm sheet of temporalis fascia was harvested, stripped of muscular tissue, and dried on a Teflon block.

Dissection was continued anteriorly to expose the posterior bony external auditory canal wall. A canal skin incision was made 6 mm lateral to the annulus, and the tympanomeatal flap was elevated anteriorly to the fibrous annulus. The annulus was elevated from the tympanic sulcus from the 6 to 12 o'clock positions, entering the middle ear space.

The middle ear was inspected under high-power operating microscope illumination: The middle ear mucosa appeared healthy and aerated without cholesteatoma. The ossicular chain was palpated: the malleus, incus, and stapes were mobile and intact with active round window reflex. The margins of the 60% central tympanic membrane perforation were denuded of squamous epithelium using a right-angle pick.

The temporalis fascia graft was trimmed, underlaid medial to the handle of the malleus and the denuded tympanic membrane remnants, and draped onto the posterior bony canal wall. The middle ear space was packed with Gelfoam pledgets soaked in ciprofloxacin otic solution to support the graft. The tympanomeatal flap was repositioned. The postauricular wound was closed in layers with 3-0 Vicryl and 4-0 Monocryl. A sterile mastoid dressing was applied.